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Correct Answer

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Practice Exam - 1

Question 31 of 117



A 22-year-old college rower presents for three-month follow-up after an episode of severe viral myocarditis. On his initial presentation he had left sided pleuritic chest pain, headache, and malaise. His high sensitivity troponin I peaked at 22.4 ng/ml, echocardiogram demonstrated LV ejection fraction of 55%, and in hospital monitoring disclosed a 5-beat run of nonsustained ventricular tachycardia. Electrocardiogram demonstrated diffuse ST-elevations. He was initiated on metoprolol tartrate 25 mg twice daily, which you discontinued two months ago. He remains on colchicine 0.6 mg twice daily. Ibuprofen was started initially and is now tapered off. He currently feels well and is eager to return to training.

What should you advise?

- A. He may return to play without restrictions
-  B. Restart metoprolol tartrate 25 mg twice daily
-  C. Obtain a stress electrocardiogram
- D. Perform cardiac magnetic resonance imaging
- E. Advise permanent restriction from training and competition

Commentary References

Testing Point: The Board-certified AHFTC specialist should know that athletes with suspected myocarditis should be restricted from training and competition for 3-6 months.

Answer: C. Obtain a stress electrocardiogram.

Myocarditis portends a risk of residual scar and arrhythmia. According to AHA/ACC Eligibility and Disqualification, athletes with myocarditis should generally avoid competitive sports for 3–6 months, with return guided by normalization of ventricular function, biomarkers, and arrhythmia testing. At 3 months, athletes should have a resting echocardiogram, a 24-hour Holter monitor, and stress electrocardiogram to evaluate for LV dysfunction or arrhythmia, whether provoked or unprovoked. Concerning findings merit further follow-up.

Thus, Answers A and E are incorrect. A cardiac MRI may be reasonable in selected circumstances where there is concern for residual scar or ongoing active myocarditis, but the stress ECG and other testing would determine that level of concern.

Answer B: In patients with full recovery, a negative Holter and no other identified sequelae, beta blocker therapy, which may impair athletic performance, are not indicated.

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